Weekly cetuximab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First cetuximab dose

Active medicines: cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before cetuximab NCCP suggested premedication.
Dexamethasone	8 mg IV	60 minutes before cetuximab NCCP suggested mandatory infusion premedication; reconcile low-risk antiemetic support to avoid duplicate steroid.
Rescue antiemetic	As individually prescribed	If required Low emetic risk; use current NCCP/local guidance and account for the infusion steroid.

Additional phase-specific support

- Skin care and sun protection under local EGFR policy.
- First two doses require post-infusion observation; delayed-reaction education.
- Monthly electrolytes/Mg/Ca for two months after last dose.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00207 v6 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-07 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required